



Memo

Attn.: All EDD & Honorable Advisors / EDDs / PSDDs / SDDs / DDs / MICs / Agency Leaders / Financial Planners (Hong Kong and Macau)

From: Toby Wong

Dept.: Product Innovation, Development & Delivery

Copy: Mr. Alger Fung / Ms. Amy Chan / Ms. Alice Liang / Ms. Melissa Wong / Mr. Louie Ho / Mr. Chris Lau / All Department Heads / Heads of Agency Division / BPs / Call Centre

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市場產品資訊速遞 – 主要危疾定義之市場比較

Market Product Information Express – Market Comparison of Key Critical Illnesses Definitions



我們就主要危疾定義與市場主要競爭對手之同類產品進行比較。下列請見總結以供參考：

- **主要危疾 (癌症、心臟病、中風及亞爾茲默氏病 / 不可還原之器質性腦退化疾病)** - 友邦之定義與市場主要競爭對手相若
- **主動脈手術** - 友邦之定義與大多數市場主要競爭對手相若
 - 以微創技術進行之主動脈手術屬早期危疾，如該危疾產品[^]涵蓋此手術，當有關索償獲批後，即可獲預支原有保額的 20%

[^]有關產品保障的範圍，請參閱產品小冊子及保單契約。

有關上述疾病/手術定義的詳細比較，請參閱附件。

如有任何查詢，請經 iEngage/PES 使用 Chatbot 進行查詢。請將以上訊息傳達予屬下的營業同事，多謝合作！



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We have conducted a market comparison on the definition of key critical illnesses and below please see a summary for your reference:

- **Key Critical Illnesses (Cancer, Heart Attack, Stroke and Alzheimer's Disease/Irreversible Organic Degenerative Brain Disorders)** – AIA's definition aligns with major competitors in the market
- **Surgery to Aorta** - AIA's definition aligns with most major competitors in the market
 - Surgery to Aorta with minimally invasive techniques is covered under Early Stage Critical Illness, if the critical illness product[^] covers this surgery, 20% of Initial Sum Assured will be payable after the claim is approved

[^]Please refer to the product brochure and policy contract for the coverage of product.

For the detailed comparison on definitions of the abovementioned illnesses / surgery, please refer to Appendix.

Should you have any queries, please enquire by using Chatbot in iEngage/PES. Please convey the above message to your fellow agents. Thanks for your attention.

Best regards,

Toby Wong
Head of Products



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附件 A) 定義比較 - 主動脈手術

	友邦	保記	宏記	安記
種類 (保額)	嚴重疾病 (100% 原有保額)			
主動脈手術	確實經 開胸或剖腹 手術進行修補或矯正主動脈瘤或主動脈阻塞、縮窄或夾層的情況。就此定義而言，「主動脈」是指胸主動脈和腹主動脈，不包括其分支血管。 血管成形術及所有其他經動脈穿刺進行的手術、導管技術、鎖孔手術或激光手術程序，均不包括在主動脈手術之內。	進行 外科手術 以矯正胸部或腹部主動脈的收窄、內壁分離或動脈瘤。	實際進行之 開胸或開腹 手術，以治療動脈瘤、主動脈縮窄、阻塞或內壁分離。 定義不包括微創性或動脈內手術，亦不包括因外傷導致之主動脈損傷。 定義中，主動脈指胸部及腹部之主動脈，惟其分支除外。	須要通過 開腹或開胸 手術切除並移植替換患病主動脈的一個部分以治療主動脈疾病。本定義中，主動脈是指胸主動脈或腹主動脈而非其分支。有關手術必須由專科醫生認為是醫學上必需的情況下進行。 保障範圍不包括治療主動脈周圍分支的血管病之手術 ，即使手術過程中主動脈的一部分被移除。
種類 (保額)	早期危疾 (20% 原有保額)			
主動脈疾病的血管介入治療	主動脈疾病的血管介入治療是指接受 微創或動脈內的手術 修補或矯正主動脈內的動脈瘤、狹窄、阻塞或夾層分離，需經相關專科的 註冊醫生證明 並提供心臟超音波檢查或任何其他適當的診斷檢驗報告。本定義中，主動脈是指胸主動脈或腹主動脈而非其分支。		主動脈疾病的血管介入治療是指接受 微創或動脈內的手術 修補或矯正主動脈內的動脈瘤、狹窄、阻塞或夾層分離。本定義中的主動脈是指胸主動脈或腹主動脈而非其分支	

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附件 B) 定義比較 – 癌

	友邦	保記	宏記	安記
癌	癌是指： (a) 任何經組織學確診為惡性之腫瘤，並須有惡性細胞已不受控制地生長並侵略其他細胞組織的特徵；或 (b) 任何經組織病理學報告證實為白血病、淋巴瘤或肉瘤。 即使上述有何規定，就「危疾」之定義而言，癌症並不包括下列任何一項： (i) 任何在組織病理學中分類為癌前病變、非侵略性、或原位癌，或邊緣性或低惡性潛力的腫瘤； (ii) 根據 TNM 評級系統，任何在組織學上被界定為 T1N0M0 或以下級別的甲狀腺腫瘤； (iii) 根據 TNM 評級系統，任何在組織學上被界定為 T1a 或 T1b 或以下級別的前列腺腫瘤； (iv) 被分類為 RAI 級別 III 以下的慢性淋巴性白血病； (v) 與人體免疫力缺乏病毒(HIV)感染同時存在的所有癌症；及 (vi) 任何非黑色素瘤的皮膚癌。	惡性腫瘤而具有惡性細胞失控的生長，並對人體組織浸潤，癌症包括白血病，但不包括以下任何情況： - 任何在組織學中分類為癌前病變、非侵略性、或原位癌，或交界性 或低惡性潛力的腫瘤； - 任何子宮頸上皮內瘤樣病變 (CIN I、CIN II 或 CIN III) 或子宮頸鱗狀上皮內病變； - 任何在人類免疫缺陷病毒 (HIV)存在下出現的腫瘤； - RAI 第 III 期以下的慢性淋巴性白血病； - 任何 惡性 黑色素瘤 以外的所有 皮膚癌； - 根據 TNM 評級系統，任何在組織學上被界定為 T1N0M0 或以下級別的甲狀腺腫瘤；及 - 根據 TNM 評級系統，任何在組織學上被界定為 T1a 或 T1b 或以下級別的前列腺腫瘤。 癌症必須經組織病理學報告確診。	須有癌細胞不受控制地生長並侵入和破壞其他正常組織的特徵的惡性腫瘤。 癌症必須獲病理學報告中提交的組織學化驗結果證實為惡性。 定義不包括下列各項： (a) 腫瘤出現原位癌惡性病變，子宮頸細胞病變、CIN-1、CIN-2、CIN-3 或組織化驗顯示腫瘤處於癌變前期； (b) 所有皮膚癌（惡性黑色素瘤除外）； (c) 經組織化驗證實屬於 TNM 分期系統中的 T1a 或 T1b 期的前列腺癌，或按其他分期系統屬同期或較早期的前列腺癌； (d) 經組織化驗證實屬於 TNM 分期系統中的 T1N0M0 或較早期的甲狀腺惡性腫瘤； (e) 尚未進入 Rai 分期第三期的慢性淋巴性白血病；及 (f) 所有感染人類免疫力缺乏症病毒後罹患的癌症。	任何經組織學首次確診為惡性之腫瘤，並須有惡性細胞已不受控制地生長並侵略其他細胞組織的特徵。惡性腫瘤包括白血病、肉瘤和淋巴瘤，但不包括皮膚淋巴瘤。 為免生疑問，以下情況並不符合癌症之定義： - 仍未入侵肌肉層的膀胱腫瘤 (Tis 及 Ta) - 任何子宮頸上皮內瘤樣病變(CIN I、CIN II 或 CIN III)或子宮鱗狀上皮內病變 - 所有經組織學分類為良性、原位癌、惡性前期、非侵入性、交界性惡性或低潛在惡性的腫瘤 - 所有存在人類免疫缺陷病毒的腫瘤 - 所有被分類為低於 RAI 第 3 期的慢性淋巴細胞白血病 - 所有經組織學分類為 TNM 分級標準低於 T2N0M0 及 Gleason 評分低於 7 的前列腺腫瘤 - 所有經組織學分類為 TNM 分級標準於 T1N0M0 或以下級別的甲狀腺腫瘤 - 所有皮膚癌，惡性黑色素瘤皮膚癌除外 - 未涉及淋巴結或其他器官的世界衛生組織 (WHO) 1 (G1) 級神經內分泌腫瘤

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附件 C) 定義比較 – 心臟病

	友邦	保記	宏記	安記
心臟病	因心臟血液供應不足，引致部份心臟肌肉（心肌）壞死，並須符合下列所有準則： (a) 典型的胸痛病歷； (b) 在相關心臟事故期間心電圖顯示新近具急性心肌梗塞特徵的變化；及 (c) 以下其中一項： (i) 心肌酵素（CPK-MB）提高至一般公認的實驗室水平的正常水平以上；或 (ii) 心肌旋轉蛋白水平達到心肌旋轉蛋白 I (Troponin I) > 0.5ng/ml 或以上，或心肌旋轉蛋白 T (Troponin T) > 1.0ng/ml 或以上。 心絞痛則明確不受此保障。	因心臟血液供應不足，引致部份心臟肌肉（心肌）壞死，並須符合下列所有準則： • 典型胸痛病歷；及 • 心肌梗塞特有的新近心電圖變化，顯示在相關心臟事故發生時有急性心肌梗塞；及 • 心臟酵素 CK-MB 有特性的上升或心肌鈣蛋白 T (Troponin T) 達到 0.6ng/ml 以上或心肌鈣蛋白 I (Troponin I) 達到 0.5ng/ml 以上。 心絞痛則明確不受此保障。	因血液供應不足而導致部份心臟肌肉（心肌）壞死。診斷必須得到下列所有準則支持，並與急性心肌梗塞相符： (a) 胸痛病歷； (b) 心電圖出現改變；及 (c) 具診斷意義的心臟酵素 CK-MB 上升，或心肌肌鈣蛋白 T > 0.6 微克/公升，或肌鈣蛋白 I > 0.5 微克/公升。 定義不包括其他所有急性冠狀綜合症，包括但不限於不穩定型心絞痛、輕微梗塞及心肌輕微受損。	因血液供應不足而導致心臟肌肉壞死及出現下列所有急性心肌梗塞的證據： (1) 心電圖顯示有典型的缺血性新變化：新的 ST-T 改變或新的左束支傳導阻滯或新的病理性 Q 波；及 (2) 心臟生化標記典型上升或心肌鈣蛋白提升及達到下列或更高水平： 2.1. 肌鈣蛋白 T (Troponin T) 高於 200ng/L (即 0.2 ng/ml 或 0.2 ug/L); 或 2.2. AccuTnI 高於 500ng/L (即 0.5 ng/ml 或 0.5 ug/L) 保障範圍不包括下列情況： - 其他急性冠狀動脈綜合症 - 沒有心肌梗塞的心絞痛 - 與心臟病發作無關的心臟酵素或心臟生化標記上升

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附件 D) 定義比較 – 中風

	友邦	保記	宏記	安記
中風	由於任何腦血管意外或事故產生神經功能性障礙，該神經功能性障礙必須持續最少四(4)個星期並從身體檢查中確定具有客觀神經異常症狀。中風包括腦組織梗塞、腦出血及由顱以外原因引致血栓塞。中風的診斷必須以電腦掃描(CT scan)或磁力共振(MRI)作證明，並必須由腦神經專科註冊醫生確定該功能性障礙。 以下各項不在受保之列： (a) 因短暫性腦缺血引致的腦部症狀； (b) 因偏頭痛引致的腦部症狀；及 (c) 對眼或視神經或前庭系統功能造成影響的血管疾病。	任何腦血管病發事件，引起神經系統後遺症持續超過24小時，包括腦組織梗塞、腦出血及源自頭顱外之栓塞。 本項疾病必須導致神經功能性受損，發病後至少4個星期由註冊腦神經科專科醫生進行身體檢查，確認有客觀神經異常症狀。 此診斷須由磁力共振掃描、電腦斷層掃描或其他可靠的顯影技術測出與新一次中風相符的結果支持。 以下情況不在受保之列： a) 短暫性腦缺血發作 (TIA)；及 b) 對眼或視覺神經或前庭系統功能造成影響的血管疾病。	因腦組織梗塞、大腦及蛛網膜下出血、腦栓塞及腦血栓等腦血管病症，導致神經系統功能上受損並須從身體檢查中確定其具有客觀的神經異常症狀，而病症必須於發生最少四個星期後經神經科專家專科醫生核實。 診斷須由磁力共振掃描、電腦斷層掃描或其他可靠的顯影技術測出與新一次中風相符的結果支持。定義不包括下列各項： (a) 短暫性腦缺血及任何可逆轉性缺血神經功能缺損； (b) 視覺神經疾病或影響眼睛的血管疾病； (c) 前庭系統缺血疾病；及 (d) 經腦部或其他掃描中發現的異常，而其異常沒有明確相關的臨床症狀和神經系統病徵。	中風指由於腦出血、腦栓塞及腦血栓而導致腦細胞死亡，及導致持續性臨床症狀的永久神經功能缺損。中風發病後至少 28 日由腦神經科專科醫生進行身體檢查，確認有清晰而明顯的感覺或運動功能異常症狀。有關事件須由磁力共振掃描(MRI)，電腦斷層掃描(CT)或其他經本公司認可之可靠的顯影技術作證明。 保障範圍不包括以下情況： - 短暫性腦缺血發作 (TIA) - 眼部血管功能障礙包括視神經或視網膜梗死 - 前庭系統的缺血性功能障礙 - 由影像學發現的無症狀性腦中風

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附件 E) 定義比較 – 亞爾茲默氏病 / 不可還原之器質性腦退化疾病

	友邦	保記	宏記	安記
亞爾茲默氏病 / 不可還原之器質性腦退化疾病	經受保人的臨床狀態及標準問卷或測驗證明受保人的思考能力退化或喪失，或行為舉止之失常是由亞爾茲默氏病或其他不可還原之器質性腦退化疾病引致，並導致受保人之思維能力及社交活動能力嚴重退減，進而影響受保人須接受持續性之護理。 亞爾茲默氏病或其他不可還原之器質性腦退化疾病的診斷必須由腦神經專科註冊醫生臨床確定。 以下所列並不包括在內： (a) 非器質性腦疾病如神經機能疾病及精神病；及 (b) 任何藥物或酒精引起的器質性腦疾病。	根據臨床狀態及認可的標準問卷或測試證實因患上阿耳滋海默氏症（腦退化性疾病）引起的智力漸進式衰退、喪失或行為異常，或不可復原的機能退化失調而導致精神及社交功能明顯減退，使受保人需要持續接受照料。 必須由註冊老年病專科醫生，註冊腦神經科專科醫生或註冊精神科專科醫生證實。 精神病、任何藥物或酒精引起的阿耳滋海默氏症/不可還原之器質性腦退化疾病及可還原的器質性腦疾病概不受保。	經臨床狀態及認可標準問卷檢查或測試，證實受保人因患上亞爾茲默氏症/不可還原之器質性腦退化性疾病（痴呆）而出現智能逐漸退化或喪失或異常行為，導致智力及社交活動能力嚴重減退，必需持續依賴他人照顧及看護。診斷必須由老人科專科醫生、神經科專科醫生或老人精神科專科醫生經臨床證實。 定義不包括因精神病、任何藥物或酒精或任何可逆轉之器質退化性腦病引致之亞爾茲默氏症及其他痴呆症。	經臨床證據及在 30 分為滿分的簡短智能測驗 (Mini Mental State Examination) 中取得 10 分以下，以證實因患上阿爾茲默氏病或其他認知障礙症引起的智力衰退或喪失或行為異常，而導致精神及社交能力明顯減退，使保受人須要持續接受照料。有關診斷必須由腦神經科或老人科專科醫生證實。 保障範圍不包括以下情況： - 非器質性腦部疾病如神經機能疾病及精神病 - 任何藥物或酒精引起的腦部疾病

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Appendix: A) Definition Comparison - Surgery to Aorta

	AIA	PRX	ManX	AXX
Category (Coverage)	Major CI (100% of Initial Sum Assured)			
Definition	The actual undergoing of surgery via a thoracotomy or laparotomy to repair or correct an aortic aneurysm, an obstruction of the aorta, a coarctation of the aorta or a dissection of the aorta. For the purpose of this definition, "aorta" shall mean the thoracic and abdominal aorta but not its branches. Angioplasty and all other intra-arterial, catheter based techniques, keyhole or laser procedures are excluded from Surgery to Aorta.	The undergoing of surgery to correct any narrowing, dissection or aneurysm of the thoracic or abdominal aorta.	The actual undergoing of laparotomy or thoracotomy to repair or correct an aneurysm, narrowing, obstruction or dissection of the aorta through surgical opening of the chest or abdomen. Surgery performed using only minimally invasive or intra-arterial techniques are excluded. Traumatic injury of the aorta is excluded. For the purpose of this definition, aorta shall mean the thoracic and abdominal aorta but not its branches.	Undergoing of a laparotomic or thoracotomic surgery to treat a disease of aorta by excision and replacement of a portion of diseased aorta with a graft. For this definition, aorta means the thoracic and abdominal aorta but not its branches. The surgery must be considered medically necessary by a Specialist. Surgery to treat peripheral vascular disease of the aortic branches is excluded even if a portion of aorta is removed during the operative procedure.
Category (Coverage)	ESCI (20% of Initial Sum Assured)			
Definition	Endovascular Treatments of Aortic Disease shall mean the undergoing of surgery via minimally invasive or intra-arterial techniques to repair or correct an aneurysm, narrowing, obstruction or dissection of the aorta, as evidenced by an echocardiogram or any other appropriate diagnostic test that is available and confirmed by a Registered Medical Practitioner who is a specialist in the relevant field. For this definition, aorta means the thoracic and abdominal aorta but not its branches.	N/A	Endovascular Treatments of Aortic Disease shall mean the undergoing of surgery via minimally invasive or intra-arterial techniques to repair or correct an aneurysm, narrowing, obstruction or dissection of the aorta. For this definition, aorta means the thoracic and abdominal aorta but not its branches	N/A

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Appendix: B) Definition Comparison - Cancer

	AIA	PRX	ManX	AXX
Cancer	Cancer means: (a) Any malignant tumour positively diagnosed with histological confirmation and characterized by the uncontrolled growth of malignant cells and invasion of tissue; or (b) Any occurrence of histologically confirmed leukemia, lymphoma or sarcoma. Irrespective of the above, for purposes of the definition of "Critical Illness", Cancer does not include any of the following: (i) any cancer which is histologically classified as pre-malignant, non-invasive, or carcinoma in situ, or as having either borderline malignancy or low malignant potential; (ii) any tumour of the thyroid histologically classified as T1N0M0 or a lower stage according to the TNM classification system; (iii) any tumour of the prostate histologically classified as T1a or T1b or a lower stage according to the TNM classification system; (iv) chronic lymphocytic leukemia classified as less than Rai Stage III; (v) any cancer where HIV Infection is also present; and (vi) any skin cancer, other than malignant melanoma.	A malignant tumour characterised by the uncontrolled growth of malignant cells and the invasion of tissue. This includes leukaemia but excludes any of the following: - any tumour which is histologically classified as pre-malignant, non-invasive, or carcinoma-in-situ, or as having either borderline malignancy or low malignant potential; - any Cervical Intra-epithelial Neoplasia (CIN I, CIN II, or CIN III) or Cervical Squamous Intra-epithelial Lesion; - any tumour in the presence of any Human Immunodeficiency Virus; - chronic lymphocytic leukaemia less than Rai Stage III; - any skin cancer other than malignant melanoma; - any thyroid tumour which is histologically classified as T1N0M0 or a lower stage according to the TNM classification system; and - any prostate tumour which is histologically classified as T1a or T1b or a lower stage according to the TNM classification system. The diagnosis of Cancer must always be confirmed by a histopathology report.	A malignant tumour, characterized by the uncontrolled growth of malignant cells with invasion and destruction of normal tissue. The cancer must be confirmed by histological evidence of malignancy on a pathology report. The following are excluded: (a) Tumours showing the malignant changes of carcinoma-in-situ, cervical dysplasia, CIN-1, CIN-2, CIN-3 or which are histologically described as pre-malignant; (b) All skin cancers other than malignant Melanomas; (c) Prostate cancers which are histologically described as TNM Classification T1a or T1b or are of another equivalent or lesser classification; (d) Malignant tumour of the thyroid which is histologically described as TNM Classification T1N0M0 or a lesser classification; (e) Chronic Lymphocytic Leukaemia less than Rai Stage 3; and (f) All tumours in the presence of Human Immunodeficiency Virus (HIV) infection.	The first unequivocal diagnosis of any malignant tumour characterised by the uncontrolled growth of malignant cells and invasion of tissue, and positively diagnosed with histological confirmation. The term malignant tumour includes leukaemia, sarcoma and lymphoma except cutaneous lymphoma (lymphoma confined to the skin) For the avoidance of doubt, the following conditions do not meet the above definition: - Urinary bladder tumours that have not invaded the muscle layer (Tis and Ta) - Cervical Intra-epithelial Neoplasia (CIN I, CIN II, or CIN III) or Cervical Squamous Intra-epithelial Lesion - All tumours which are histologically classified as benign, carcinoma-in-situ, pre-malignant, non-invasive, having borderline malignancy or having low malignant potential - All tumours in the presence of any human immunodeficiency virus - All chronic lymphocytic leukaemia classified as less than Rai stage III - All prostate tumours which are histologically classified as less than T2N0M0 according to the TNM classification system and having a Gleason score below 7 - All thyroid tumours which are histologically classified as T1N0M0 or a lower stage according to the TNM classification system - All skin tumours except malignant melanoma - WHO Grade 1 neuroendocrine tumours without lymph node or other organ involvement

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Appendix: C) Definition Comparison – Heart Attack

	AIA	PRX	ManX	AXX
Heart Attack	The death of a portion of the heart muscle (myocardium) as a result of inadequate blood supply, where all of the following criteria are met: (a) A history of typical chest pain; (b) New characteristic ECG changes indicating acute myocardial infarction at the time of the relevant cardiac incident; and (c) Either (i) elevation of cardiac enzymes (CPK-MB) at levels above the generally accepted laboratory levels of normal, or (ii) troponins recorded at a level of Troponin I > 0.5ng/ml or higher, or at a level of Troponin T > 1.0ng/ml or higher. Angina is specifically excluded.	The death of a portion of the heart muscle (myocardium) as a result of inadequate blood supply, where all of the following criteria are met: - a history of typical chest pain; and - new characteristic ECG changes indicating acute myocardial infarction at the time of the relevant cardiac incident; and - the characteristic rise of cardiac enzymes CK-MB or troponin T > 0.6ng/ml or troponin I > 0.5ng/ml. Angina is specifically excluded.	Death of a portion of the heart muscle, (myocardium) arising from inadequate blood supply to the relevant area. The diagnosis must be supported by all of the following criteria and be consistent with acute myocardial infarction: (a) A history of chest pain; (b) New electrocardiographic changes, and (c) Diagnostic elevation of cardiac enzyme CK-MB or cardiac troponin T > 0.6 mcg/L or Troponin I > 0.5mcg/L. All other acute coronary syndromes, including, but not limited to, unstable angina, microinfarction and minimal myocardial damage are excluded.	Death of heart muscle due to inadequate blood supply that has resulted in all of the following evidence of acute myocardial infarction: (1) New typical ischaemic changes in the electrocardiograph: new ST–T changes or new left bundle branch block or new pathological Q waves; and (2) The characteristic rise of cardiac biomarkers or Troponins recorded at the following levels or higher: 2.1. Troponin T > 200ng/L (0.2 ng/ml or 0.2 ug/L); or 2.2. AccuTnI > 500ng/L (0.5 ng/ml or 0.5 ug/L) The following are excluded: - Other acute coronary syndromes - Angina without myocardial infarction - Non-heart attack related causes of elevated cardiac enzymes or biomarkers

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Appendix: D) Definition Comparison – Stroke

	AIA	PRX	ManX	AXX
Stroke	Any cerebrovascular accident or incident producing neurological functional impairment, with objective neurological abnormal signs on physical examination, lasting at least four (4) weeks. Infarction of brain tissue, haemorrhage and embolism from an extra-cranial source are included. The Diagnosis of Stroke must be based on changes seen in a CT scan or MRI and such functional impairment must be confirmed by a Registered Medical Practitioner who is a neurologist. The following are excluded: (a) Cerebral symptoms due to transient ischaemic attacks; (b) Cerebral symptoms due to migraine; and (c) Vascular disease affecting the eye or optic nerve or vestibular functions.	Any cerebrovascular incident producing neurological sequelae lasting more than 24 hours and including infarction of brain tissue, haemorrhage and embolisation from an extra-cranial source. This event must result in neurological functional impairment with objective neurological abnormal sign on physical examination by a Registered Specialist Neurologist at least 4 weeks after the event. The diagnosis must be supported by findings on Magnetic Resonance Imaging, Computerized Tomography, or other reliable imaging techniques consistent with the diagnosis of a new Stroke.	A cerebrovascular incident including infarction of brain tissue, cerebral and subarachnoid haemorrhage, cerebral embolism and cerebral thrombosis resulting in functional neurological impairment with objective neurological abnormal signs on physical examination which must be confirmed by a Specialist Medical Practitioner who is a neurologist at least four weeks after the event. The diagnosis must be supported by findings on Magnetic Resonance Imaging, Computerized Tomography, or other reliable imaging techniques consistent with the diagnosis of a new Stroke. The following are excluded: (a) Transient Ischaemic Attacks and any Reversible Ischaemic Neurological Deficit; (b) Disorders of the optic nerve or of the blood vessels affecting the eye; (c) Ischaemic disorders of the vestibular system; and (d) An abnormality seen on brain or other scans without clearly related clinical symptoms and neurological signs.	Stroke means the death of brain tissue caused by haemorrhage, embolism or thrombosis resulting in permanent neurological deficit with persistent clinical symptoms. There must be clear and obvious abnormalities of sensory or motor functions during the physical examination performed by a Specialist in neurology at least 28 days after the onset of stroke. The incident must be demonstrated by Magnetic Resonance Imaging (MRI), Computerised Tomography (CT) or other reliable imaging techniques approved by the Company. The following are excluded: - Transient Ischaemic Attacks (TIA) - Disorders of the blood vessels affecting the eye including infarction of the optic nerve or retina - Ischaemic disorders of the vestibular system - Asymptomatic silent stroke found on imaging

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Appendix: E) Definition Comparison – Alzheimer's Disease / Irreversible Organic Degenerative Brain Disorders

	AIA	PRX	ManX	AXX
Alzheimer's Disease / Irreversible Organic Degenerative Brain Disorders	Deterioration or loss of intellectual capacity or abnormal behaviour, as evidenced by the Insured's clinical state and accepted standardized questionnaires or tests, arising from Alzheimer's Disease or other irreversible organic degenerative brain disorder, which results in significant reduction in the Insured's mental and social functioning such that continuous supervision of the Insured is required. The Diagnosis of Alzheimer's Disease or other irreversible organic degenerative brain disorder must be clinically confirmed by a Registered Medical Practitioner who is a neurologist. The following are excluded: (a) non-organic brain disorders such as neurosis and psychiatric illnesses; and (b) drug or alcohol related organic brain disorder.	Progressive deterioration or loss of intellectual capacity or abnormal behaviour as evidenced by the clinical state and accepted standardised questionnaires or tests arising from Alzheimer's Disease or irreversible organic degenerative disorders resulting in significant reduction in mental and social functioning requiring the continuous supervision of the Life Assured. The diagnosis must be confirmed by a Registered Specialist Geriatrician, Registered Specialist Neurologist or Registered Specialist Psychiatrist. Alzheimer's disease or other dementia caused by psychiatric illness, any drug or alcohol use or any reversible organic brain disorder is excluded.	Progressive deterioration or loss of intellectual capacity or abnormal behaviour as evidenced by the clinical state and accepted standardised questionnaires or tests arising from Alzheimer's Disease or irreversible organic degenerative brain disorders, resulting in significant reduction in mental and social functioning requiring the continuous care and supervision of the Life Insured. The diagnosis must be clinically confirmed by a Specialist Medical Practitioner who is a geriatrician, neurologist or geriatric psychiatrist. Alzheimer's disease or other dementia caused by psychiatric illness, any drug or alcohol use or any reversible organic brain disorder is excluded.	Deterioration or loss of intellectual capacity or abnormal behaviour, as evidenced by the Insured's clinical state and a Mini Mental State Examination score of less than 10 out of 30, arising from Alzheimer's disease or other dementias, which results in significant reduction in the Insured's mental and social functioning such that continuous supervision of the Insured is required. The diagnosis must be clinically confirmed by a Specialist in neurology or geriatrics. The following are excluded: - non-organic brain disorders such as neurosis and psychiatric illnesses - drug or alcohol related organic brain disorder

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